

EMS review: quick guide

10 fictional EMS reports created with AI—not recordings of real patients.

Annotation workbook: [📄 EMS Clinician Annotation — Synthetic Pilot](#)

Start with three cases

Open Doctor review. Fill only the yellow boxes for Cases 1–3, without seeing our AI's answers. Then discuss the questions and preparation choices with us before doing the rest.

Brief ED handoff

What should the ED know? Write 2–3 sentences, as you would in a handoff. Use only the reported information; keep important uncertainty. Missing information does not mean a normal finding.

Important change?

Choose Yes, No, Unclear, or No earlier report. Improvement can matter too. In the box below, give a short reason using the reported findings and note any important missing information. If none, write “None.”

Preparation needed?

- Prepare now — preparation or escalation of awareness should start without delay.
- Can wait — preparation matters, but more time-sensitive patients can come first.
- Routine — keep the patient on the radar; no special preparation need is established.
- Unsure — not enough reliable information to judge.

These are draft preparation choices, not validated triage scores. No diagnosis or treatment plan is required.

When finished

Choose Done, or Question for team if something is unclear. Flag unrealistic reports. The team handles the technical paperwork.

Optional: compare three patients

On Queue exercise, rank each group: 1 means prepare first. Ties are allowed; choose Unsure when needed. Use the stated arrival times and resources, and briefly explain your order.

Your review gives us an expert reference for fictional cases—not proof of safety or effectiveness in real care. Clinicians retain responsibility for clinical decisions.